Vitera Health

Catching decompensation before the hospital does

Seed Round | October 2026

EUR 180K	11	3,400	CE IIa
ARR	Clinics	Patients monitored	Regulatory status

The Problem

Heart failure patients deteriorate for days before anyone notices.

1 in 4

Heart failure patients are readmitted within 30 days of discharge.

EUR 2.4B

Spent each year by EU health systems on avoidable HF readmissions.

90 days

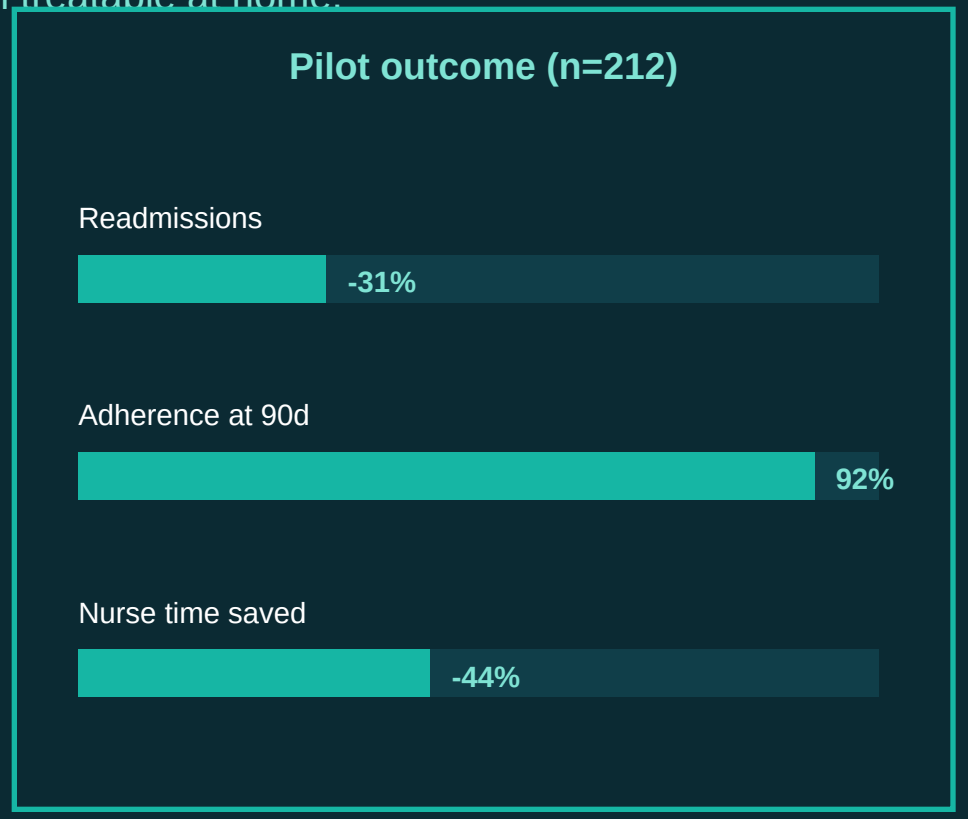
Typical gap between outpatient cardiology appointments.

Decompensation is visible in weight and vitals 7-10 days early. Nobody is watching.

Our Solution

A CE-marked remote monitoring service that flags deterioration while it is still treatable at home.

1. Home kit: cuff-free blood pressure, weight, and a 30-second symptom check
2. Risk model trained on 1.2M patient-days of longitudinal HF data
3. Nurse triage dashboard that ranks the day's patients by deterioration risk
4. CE Class IIa, ISO 13485 quality system, GDPR-compliant EU hosting



How It Works

01

Enrol

Clinic enrols a patient in under 10 minutes. Kit ships next day.

02

Monitor

Patient takes daily readings at home. Takes about 90 seconds.

03

Predict

Risk model scores every patient each morning and ranks the queue.

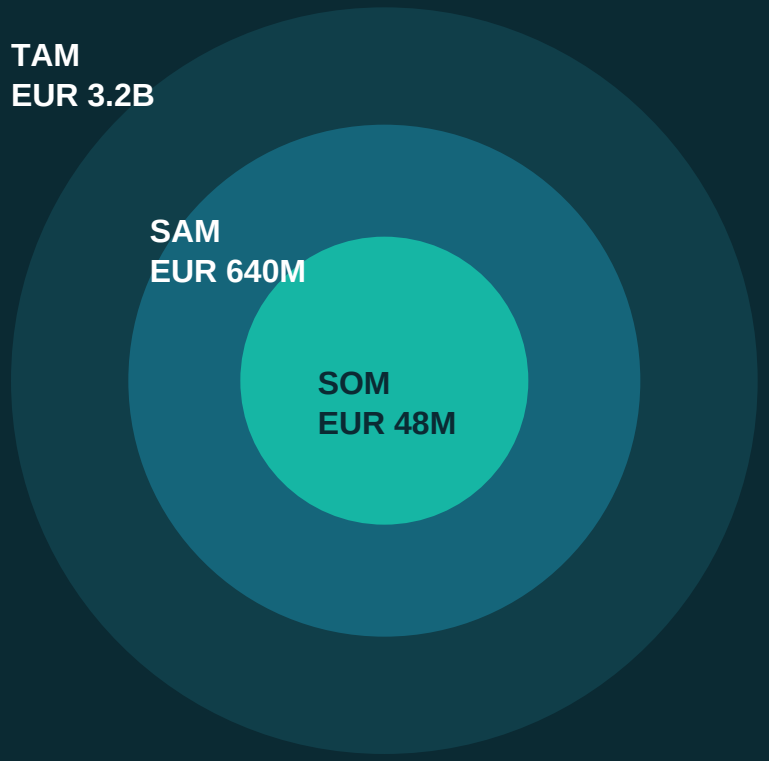
04

Intervene

Nurse calls, titrates medication, escalates only when needed.

Reimbursed in Germany under the DiGA pathway since Q2 2026.

Market Opportunity



15M people live with heart failure in Europe

9 EU countries now reimburse remote patient monitoring

EUR 2.1B German DiGA spend forecast for 2028

Business Model

Clinic

EUR 39

per patient / month

+ Nurse triage dashboard
+ Up to 150 patients
+ Email support
+ Standard reporting

MOST COMMON

Payer / DiGA

EUR 89

per patient / month

+ Reimbursed prescription
+ Unlimited patients
+ Dedicated clinical lead
+ Outcome reporting

Hospital network

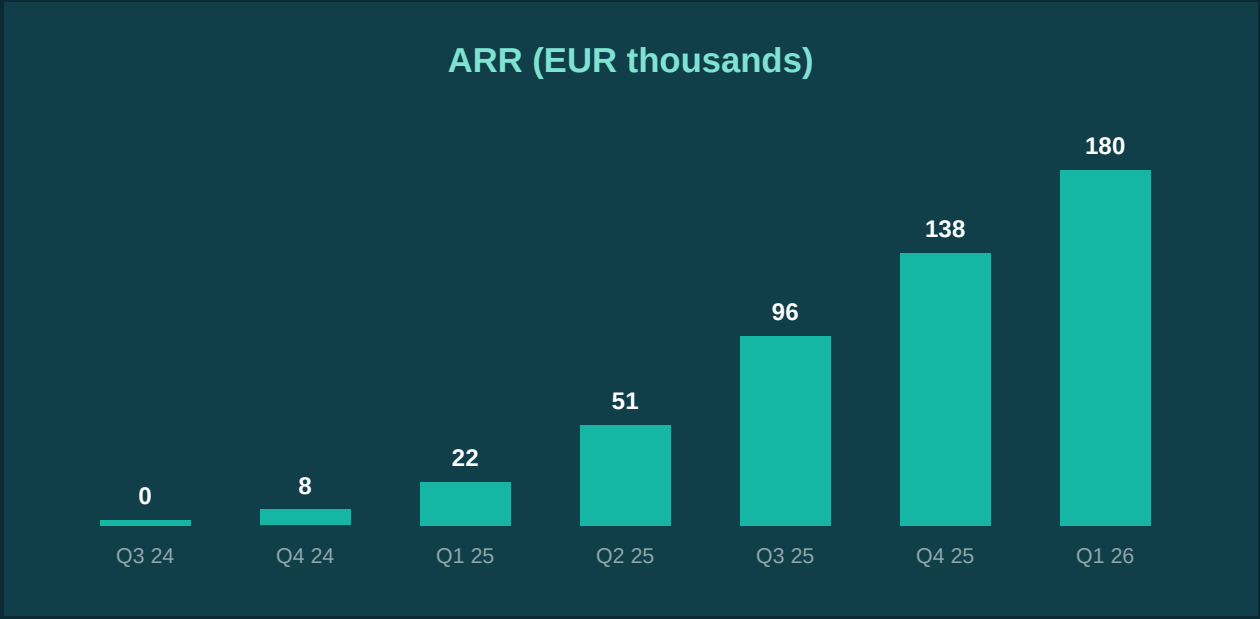
Custom

annual licence

+ Multi-site deployment
+ EHR integration
+ SLA guarantee
+ Custom risk thresholds

Gross margin 74% | Average contract value EUR 31K | CAC payback 14 months | Net retention 118%

Traction



11	Clinics live
3,400	Patients monitored
92%	90-day adherence
118%	Net revenue retention

Clinical partners:

- Klinikum Nordstadt
- Rheinland Health Group
- Praxisverbund Sued
- St. Anna Cardiology

The Team

LH

Dr. Lena Hofmann

CEO & Co-founder

Cardiologist, 9 years clinical practice. PhD digital health, TU Munich.

TF

Tomas Ferreira

CTO & Co-founder

ML lead for implantable diagnostics. 2 patents in physiological signal processing.

AK

Anja Kowalski

Head of Clinical & Regulatory

Led two CE Class IIa submissions. Former notified-body auditor.

JB

Jonas Brandt

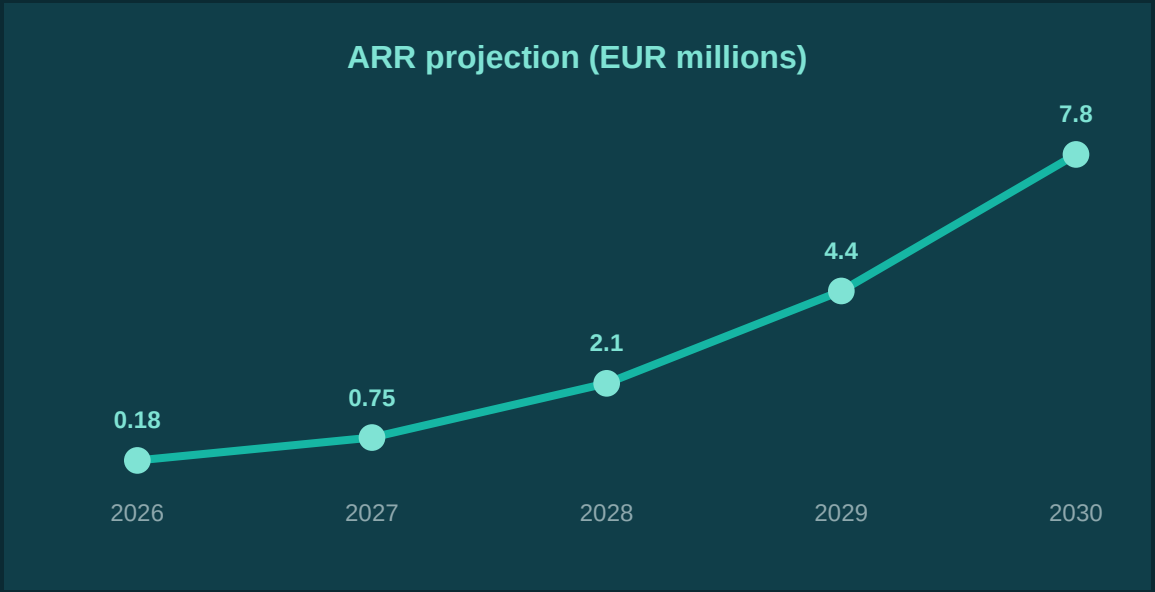
Head of Commercial

Built DACH sales for a reimbursed digital therapeutic from 0 to EUR 4M ARR.

Clinical advisory board:

Professor of Cardiology, university hospital DACH | Former head of digital health, statutory health insurer | Health economist, LSE

Financials & Roadmap



Q1 2027
DiGA permanent listing | 400-patient validation study starts

Q3 2027
50 clinics live | EUR 750K ARR

Q2 2028
Benelux market entry | EHR integration GA

Q4 2028
Break-even | EUR 2.1M ARR

Burn rate EUR 38K/month | 16-month runway post-raise | Use of funds: clinical evidence 35%, product 30%, regulatory 20%, go-to-market 15%

Keeping heart failure patients out of hospital, one morning reading at a time.

Raising EUR 450K Seed

to complete the DiGA listing, run the 400-patient validation study, and reach 50 clinics

Let's talk

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